



MIST- MEDICAL INSTITUTE FOR SCREENING TEST

MOCK TEST 4 PAPER 2 DATED ON 7th JUNE 2026

QUESTIONS

- Q1. A 60-year-old patient with a 20-year history of metabolic syndrome and poor glycemic control presents with acute-on-chronic ocular hypertension (50 mmHg) and corneal microcystic edema. Slit-lamp biomicroscopy reveals fine, tufted vascularization at the pupillary border and an anterior chamber angle occluded by a fibrovascular membrane. Fundoscopy shows an increased cup-to-disc ratio and generalized retinal ischemia. Which of the following is the most likely diagnosis?
- a) Pigmentary glaucoma
 - b) Neovascular glaucoma
 - c) Malignant glaucoma
 - d) Acute primary angle-closure glaucoma
- Q2. A 25-year-old female presents with anisocoria and subjective photophobia. The right pupil is 6 mm and the left is 3 mm in ambient light. The right pupil demonstrates a segmental, vermiform contraction of the iris sphincter under high magnification and exhibits a significant "light-near dissociation." The patient mentions a nonspecific febrile illness three weeks prior. Which pharmacological agent would most likely confirm the site of the lesion?
- a) 4% Cocaine
 - b) 1% Apraclonidine
 - c) 0.125% Pilocarpine
 - d) 1% Atropine
- Q3. Following a high-velocity blunt force injury to the globe, a patient presents with diplopia on vertical saccades and infraorbital hypoesthesia. Computed Tomography (CT) reveals herniation of orbital fat and the inferior rectus muscle into the maxillary sinus. This clinical triad is most indicative of which pathology?
- a) Traumatic optic neuropathy
 - b) Superior orbital fissure syndrome
 - c) Orbital blow-out fracture
 - d) Cavernous sinus thrombosis
- Q4. A patient who underwent a keratorefractive procedure (photoablative flattening) a decade ago is now undergoing biometry for senile cataract extraction. The surgeon notes that the "Standard Keratometry" method is likely to result in a "hyperopic surprise" (underestimation of IOL power). Which parameter has been most significantly altered, leading to this calculation error?
- a) Vitreous chamber depth
 - b) Relationship between anterior and posterior corneal curvature
 - c) Crystalline lens thickness
 - d) Axial length of the globe
- Q5. A postoperative patient presents with a "hypopyon", intense vitritis, and loss of the red reflex. The sclera remains white and quiet, and there is no significant limitation of extraocular motility or proptosis. Which of the following describes the anatomical extent of this condition?
- a) Inflammation of the entire globe and Tenon's capsule
 - b) Inflammation confined to the uveal tract, retina, and vitreous
 - c) Idiopathic inflammation of the extraocular muscles
 - d) Suppurative infection of the cornea only



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- Q6. A patient presents with chronic ocular surface irritation without any primary malposition of the eyelid margins. Slit-lamp biomicroscopy demonstrates aberrant pilosebaceous differentiation at the posterior lamella of the eyelid, with accessory cilia emerging from glandular orifices. What is the most appropriate diagnosis?
- Ectropion
 - Entropion
 - Distichiasis
 - Trichiasis
- Q7. An adult presents with progressive, symmetric anterior displacement of the globes associated with restrictive ophthalmopathy. Histopathology of orbital tissue would most likely demonstrate which of the following processes?
- Malignant mesenchymal proliferation
 - Acute suppurative inflammation of orbital septum
 - Autoimmune-mediated fibroblast activation with glycosaminoglycan deposition
 - Monoclonal lymphoid infiltration
- Q8. A patient presents with an acute rise in intraocular pressure due to impaired aqueous humor outflow at the level of the iridocorneal angle. Immediate pharmacologic intervention is planned using a systemic agent that reduces vitreous volume via osmotic gradient. Which of the following is the most appropriate initial therapy?
- Muscarinic antagonist
 - Hyperosmotic agent
 - Topical antimicrobial
 - Corticosteroid preparation
- Q9. In an infiltrative orbitopathy characterized by restrictive extraocular myopathy, selective involvement of certain muscles leads to predictable gaze limitation patterns. Based on typical disease progression, which extraocular muscle is most frequently and earliest affected?
- Medial rectus
 - Superior rectus
 - Inferior rectus
 - Inferior oblique
- Q10. A 22-year-old male presents to the emergency department following blunt periorbital trauma. Clinical evaluation reveals restrictive strabismus on supraduction and relative enophthalmos. Neurological assessment demonstrates ipsilateral anesthesia in the distribution of the maxillary division of the trigeminal nerve, specifically over the malar prominence and upper lip. Which of the following is the most likely diagnosis?
- Lenticular rosette degeneration
 - Traumatic optic neuropathy
 - Orbital blow-out fracture
 - Traumatic vitreous hemorrhage
- Q11. A neonate is evaluated for an abnormal palpebral fissure. Slit-lamp biomicroscopy reveals fine, inextensible fibrovascular bands tethering the ciliary margins of the superior and inferior palpebrae. The bulbar and palpebral conjunctivae, as well as the corneal epithelium, remain anatomically distinct and entirely uninvolved. Which of the following is the correct designation for this congenital anomaly?



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- a) Symblepharon
- b) Ankyloblepharon
- c) Cicatricial entropion
- d) Distichiasis

Q12. A 28-year-old patient presents with a sudden onset of profound monocular vision loss and dyschromatopsia. Clinical evaluation reveals a positive Marcus Gunn phenomenon in the affected eye. However, dilated fundusoscopic examination demonstrates a pristine optic disc with distinct margins and a normal cup-to-disc ratio, as well as a healthy retinal vasculature. Which of the following is the most likely pathological localization for this presentation?

- a) Retrobulbar neuritis
- b) Central retinal artery occlusion
- c) Papillitis
- d) Neuroretinitis

Q13. A 60-year-old male with a history of high myopia corrected via photoablative keratomileusis ten years prior is scheduled for phacoemulsification. The surgeon notes that utilizing standard paraxial keratometry is likely to result in an erroneous underestimation of the required intraocular lens (IOL) power, potentially leading to a "hyperopic surprise" postoperatively. This calculation error is primarily attributed to which of the following?

- a) Alteration of the vitreous chamber axial length
- b) Invalidation of the keratometric refractive index
- c) Progressive lenticular astigmatism
- d) Posterior staphyloma progression

Q14. A patient presents with acute-onset ocular pain and a "cloudy" vitreous three days following an uncomplicated cataract extraction. Clinical examination reveals a significant hypopyon and fibrinoid reaction in the anterior chamber. The clinician determines that the inflammatory process is confined to the intraocular compartments and has not breached the external fibrous tunic of the eye. Which of the following structures is spared in this specific inflammatory state?

- a) Sclera
- b) Uveal tract
- c) Neurosensory retina
- d) Vitreous body

Q15. A patient is diagnosed with an acute congestive episode of primary angle closure in the right eye. Upon stabilization of the acute crisis, the ophthalmologist focuses on the contralateral eye, which exhibits a narrow iridocorneal angle on gonioscopy. To definitively eliminate the risk of a pupillary block mechanism in this "fellow eye," which of the following is the most appropriate prophylactic intervention?

- a) Topical corticosteroid therapy
- b) Surgical trabeculectomy
- c) Topical prophylactic antibiotics
- d) Laser peripheral iridotomy

Q16. A 58-year-old postmenopausal female on exogenous estrogen therapy presents with a non-tender, fixed, 3 cm scirrhous lesion in the upper outer quadrant of the left mammary gland. Mammographic evaluation demonstrates clustered pleomorphic microcalcifications and a spiculated mass. While triple assessment is being completed, the surgeon necessitates a diagnostic modality that facilitates the assessment



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of histological grade, lymphovascular invasion, and immunohistochemical profiling for hormone receptor expression. Which of the following is the most appropriate definitive investigation?

- a) Fine-needle aspiration cytology
- b) Automated core needle biopsy
- c) Digital breast tomosynthesis
- d) Vacuum-assisted excision

Q17. A 35-year-old male presents with a non-tender, fluctuant paratesticular mass. Physical examination reveals that the mass is situated superior to the gonad, and the clinician is able to "get above the swelling." The lesion demonstrates brilliant transillumination, and the testis is palpated as a distinct entity from the cystic structure. This pathology represents a retention cyst of which of the following anatomical structures?

- a) Tunica vaginalis
- b) Pampiniform plexus
- c) Vasa efferentia
- d) Processus vaginalis

Q18. A 62-year-old patient undergoes a segmental colectomy for an adenocarcinoma of the descending colon. The surgical specimen includes a wide lymphadenectomy. The pathological report indicates that five regional lymph nodes are positive for metastatic deposits, though no distant visceral metastasis is detected. According to the AJCC TNM staging system, which nodal category is assigned to this patient?

- a) N1a
- b) N1b
- c) N2a
- d) N2b

Q19. A 40-year-old patient presents with acute proctalgia exacerbated by defecation. During the differential diagnosis of anorectal pathologies, the clinician distinguishes between conditions based on their sensory innervation. Which of the following conditions is typically characterized by a painless clinical course due to its origin superior to the pectinate line?

- a) Fissure-in-ano
- b) Perianal abscess
- c) Internal hemorrhoids
- d) Proctalgia fugax

Q20. A 68-year-old male undergoes a transurethral resection of the prostate (TURP) for refractory lower urinary tract symptoms. Postoperatively, the patient is monitored for "TURP syndrome." If the surgeon utilized a monopolar resection system with glycine irrigation, the physiological basis of this syndrome is primarily:

- a) Dilutional hyponatremia and fluid overload
- b) Dilutional hyponatremia and glycine toxicity
- c) Hyperglycemia-induced osmotic diuresis
- d) Acute hyperkalemia from prostatic tissue necrosis

Q21. A 25-year-old male presents with a protracted course of crampy lower abdominal pain and hematochezia. Endoscopic evaluation reveals a symmetrical, circumferential inflammatory process of the colonic mucosa extending proximally from the anal verge without intervening healthy tissue. Histopathological analysis of the mucosal biopsies demonstrates marked goblet cell depletion and the



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presence of neutrophilic aggregates within the glandular lumina. Which of the following is the most likely diagnosis?

- a) Regional enteritis
- b) Ischemic colitis
- c) Ulcerative colitis
- d) Pseudomembranous colitis

Q22. A 30-year-old female presents with acute-onset right hypochondrial pain and pyrexia. The patient exhibits a reflexive cessation of inspiratory effort upon deep palpation of the right subcostal margin. The pain is noted to refer to the ipsilateral supraclavicular region. Which of the following is the primary pathophysiological mechanism responsible for this presentation?

- a) Obstruction of the cystic duct leading to gallbladder wall ischemia
- b) Transmural migration of pancreatic enzymes into the lesser sac
- c) Perforation of a prepyloric ulcer into the peritoneal cavity
- d) Obstruction of the common bile duct resulting in ascending cholangitis

Q23. A 22-year-old primiparous woman in her third week of lactation presents with localized mammary erythema, induration, and exquisite tenderness centered around the areolar complex. Physical examination reveals cutaneous denudation of the nipple. Which of the following is the most likely etiologic agent responsible for this suppurative process?

- a) Streptococcus pyogenes
- b) Staphylococcus aureus
- c) Bacteroides fragilis
- d) Corynebacterium minutissimum

Q24. A 24-year-old male is brought to the trauma bay following a high-impact cephalic injury. The patient initially regained consciousness but subsequently experienced a rapid decline in his Glasgow Coma Scale (GCS) score. A non-contrast cranial CT scan demonstrates a lenticular, hyperdense extra-axial collection that is restricted by the cranial sutures. The most likely source of this hemorrhage is:

- a) Rupture of the bridging cortical veins
- b) Laceration of the middle meningeal artery
- c) Rupture of a berry aneurysm in the Circle of Willis
- d) Shearing of the long white matter axons

Q25. A 42-year-old euthyroid female presents with a solitary, firm thyroid nodule. Ultrasonographic evaluation reveals a solid, hypoechoic lesion with indistinct margins and clustered psammomatous microcalcifications. Which of the following is the most appropriate next step to establish a cytological diagnosis according to the Bethesda System?

- a) Quantitative serum thyroglobulin assay
- b) Technetium-99m pertechnetate scintigraphy
- c) Ultrasound-guided fine-needle aspiration
- d) Positron emission tomography (PET) scan

Q26. A 50-year-old female presents with recurrent nephrolithiasis and osteopenia. Biochemical analysis reveals hypercalcemia, hypophosphatemia, and an inappropriately elevated serum parathyroid hormone (PTH) level. Localization studies confirm a solitary hyperfunctioning lesion of the inferior parathyroid gland. What is the definitive management for this patient's metabolic derangement?

- a) Targeted surgical excision of the adenoma



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- b) Chronic therapy with second-generation bisphosphonates
- c) Administration of calcimimetic agents
- d) Aggressive loop diuretic therapy and saline hydration

Q27. A 30-year-old male presents with paroxysmal hypertension, diaphoresis, and tremors. Biochemical screening reveals elevated fractionated plasma metanephrines. Family history is significant for medullary thyroid carcinoma. Genetic testing identifies a germline mutation in the RET proto-oncogene. Which of the following constellations of neoplasms is most likely to be present in this patient?

- a) Parathyroid hyperplasia, Pancreatic islet cell tumors, Pituitary adenoma
- b) Medullary thyroid carcinoma, Pheochromocytoma, Parathyroid hyperplasia
- c) Medullary thyroid carcinoma, Pheochromocytoma, Mucosal neuromas
- d) Papillary thyroid carcinoma, Pheochromocytoma, Parathyroid adenoma

Q28. A 35-year-old male is diagnosed with a catecholamine-secreting tumor of the adrenal medulla. To ensure hemodynamic stability during the intraoperative period and prevent a hypertensive crisis during tumor manipulation, which of the following pharmacological protocols must be strictly followed?

- a) Immediate initiation of non-selective beta-blockade
- b) Alpha-adrenergic blockade followed by cautious beta-blockade
- c) Intravenous corticosteroid replacement therapy
- d) Preoperative administration of ACE inhibitors and diuretics

Q29. Immediately following a total thyroidectomy, a patient develops inspiratory stridor and acute respiratory distress upon extubation. Fiberoptic laryngoscopy reveals that both vocal folds are fixed in a paramedian position during the respiratory cycle. This life-threatening complication is most likely due to the bilateral injury of which nerve?

- a) Superior laryngeal nerve (external branch)
- b) Recurrent laryngeal nerve
- c) Ansa cervicalis
- d) Glossopharyngeal nerve

Q30. A 22-year-old patient presents with a painless, fluctuant mass located along the anterior border of the sternocleidomastoid muscle at the level of the mandibular angle. Fine-needle aspiration of the cystic contents reveals a straw-colored fluid characterized by the presence of numerous cholesterol crystals. This lesion most likely represents a developmental remnant of which embryological structure?

- a) First branchial pouch
- b) Second branchial cleft
- c) Thyroglossal duct
- d) Third pharyngeal arch

Q31. A 28-year-old patient presents with a chronic, non-tender midline cervical mass situated infrahyoidally. Clinical assessment reveals that the lesion ascends during deglutition but remains stationary during forced glossal protrusion. The mass is soft and lacks any associated bruit or lymphadenopathy. Which of the following is the most likely pathological entity?

- a) Thyroglossal duct cyst
- b) Plunging ranula
- c) Subhyoid bursa
- d) Delphian lymphadenopathy



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- Q32. A 65-year-old male presents with chronic halitosis, nocturnal cough, and gurgling sounds in the neck during deglutition. Barium esophagram demonstrates a contrast-filled sac originating through a zone of diminished muscular resistance between the thyropharyngeus and cricopharyngeus fibers. This condition is primarily classified as:
- Traction diverticulum
 - Pulsion diverticulum
 - Epiphrenic diverticulum
 - Intramural pseudodiverticulosis
- Q33. A patient is brought to the trauma center following a high-deceleration motor vehicle accident. The individual is in a state of persistent vegetative coma, yet the initial non-contrast computed tomography (NCCT) of the head reveals no significant intracranial hematoma, midline shift, or mass effect. However, the presence of punctate hemorrhages in the corpus callosum is noted. Which of the following mechanisms is responsible for this clinicoradiological dissociation?
- Focal contrecoup contusion
 - Rotational acceleration-induced shearing
 - Rupture of bridging veins in the subdural space
 - Laceration of the middle meningeal artery
- Q34. During an elective thoracotomy for a patient with chronic obstructive pulmonary disease, the anesthesiologist notes a sudden loss of pulmonary compliance, a precipitous drop in end-tidal CO_2 , and ipsilateral absence of breath sounds. The patient develops profound hypotension and a deviated trachea. Which of the following is the most immediate life-saving intervention?
- Intravenous administration of high-dose corticosteroids
 - Immediate needle decompression in the second intercostal space
 - Pericardiocentesis via the subxiphoid approach
 - Emergency fiberoptic bronchoscopy
- Q35. A 58-year-old male presents with the classic triad of intermittent claudication of the hip and gluteal muscles, absent femoral pulses, and erectile dysfunction. This constellation of symptoms, known as Leriche Syndrome, is indicative of chronic occlusive disease at which anatomical site?
- Adductor canal
 - Popliteal artery
 - Aortoiliac bifurcation
 - Profunda femoris artery
- Q36. A postoperative patient exhibits a positive Homan's sign and localized calf tenderness. The clinician suspects a condition governed by Virchow's Triad. Which of the following diagnostic modalities is considered the most appropriate initial investigation to confirm the presence of an intravascular thrombus?
- Impedance plethysmography
 - Contrast venography
 - Color Doppler duplex ultrasonography
 - D-dimer assay
- Q37. A biopsy of a well-defined, 1.5 cm 1.5 cm lesion at the anal verge (distal to the pectinate line) confirms a keratinizing squamous cell carcinoma. There is no evidence of sphincter invasion or regional lymphadenopathy. What is the standard oncological management for this specific lesion?
- Nigro protocol (chemoradiotherapy)



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- b) Abdominoperineal resection (APR)
- c) Wide local excision with 1 cm margins
- d) Proctocolectomy

Q38. A 45-year-old female is diagnosed with a 2 cm x 2 cm infiltrating ductal carcinoma in the upper outer quadrant of the breast. There is no clinical evidence of axillary lymphadenopathy or distant metastasis. Which surgical approach, when combined with adjuvant radiotherapy, is currently considered to offer survival rates equivalent to a total mastectomy?

- a) Skin-sparing mastectomy
- b) Breast-conserving surgery (lumpectomy)
- c) Modified radical mastectomy
- d) Subcutaneous mastectomy

Q39. A patient with a history of alcohol abuse presents with hematemesis. Upper gastrointestinal endoscopy reveals a non-transmural, longitudinal mucosal laceration at the squamocolumnar junction. The lesion is not actively bleeding at the time of the procedure. What is the most appropriate management strategy?

- a) Endoscopic band ligation
- b) Sengstaken-Blakemore tube insertion
- c) Supportive care with acid suppression
- d) Urgent total gastrectomy

Q40. A 3-week-old male infant presents with non-bilious, projectile emesis and signs of dehydration. Physical examination reveals an "olive-like" mass in the right upper quadrant and visible gastric peristaltic waves. Which of the following biochemical profiles is most characteristic of this condition?

- a) Hyperchloremic metabolic acidosis
- b) Hypochloremic, hypokalemic metabolic alkalosis
- c) Hyperkalemic metabolic alkalosis
- d) Normal electrolytes with respiratory acidosis

Q41. A 3-week-old neonate presents with repetitive, non-bilious emesis. Clinical examination reveals a firm, mobile mass in the epigastric region and visible gastric hyperperistalsis. Laboratory analysis confirms a metabolic alkalosis with paradoxical aciduria. Which of the following ultrasonographic criteria is considered the gold standard for confirming the diagnosis?

- a) Pyloric muscle thickness greater than 3 mm
- b) Pyloric canal length less than 10 mm
- c) Presence of a "target sign" in the transverse plane of the mid-gut
- d) Failure of the gastroesophageal sphincter to relax on swallowing

Q42. A 45-year-old female presents with symptoms suggestive of acute cholecystitis. Initial transabdominal ultrasonography fails to demonstrate cholelithiasis or sludge; however, the patient remains febrile with localized peritonitis in the right hypochondrium. A hepatobiliary iminodiacetic acid (HIDA) scan is performed, demonstrating prompt hepatic uptake and excretion of the radiotracer into the duodenum, but persistent non-visualization of the gallbladder at 4 hours. What is the most likely diagnosis?

- a) Biliary dyskinesia
- b) Acute acalculous cholecystitis
- c) Chronic cholecystitis with porcelain gallbladder
- d) Choledocholithiasis



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- Q43. A 20-year-old male sustains a straddle injury to the perineum. Clinical examination reveals a "butterfly" pattern of ecchymosis involving the scrotum and perineum, and blood is noted at the external urethral meatus. If the injury involves a rupture of the bulbar urethra and the overlying Buck's fascia, extravasated urine will most likely be confined by which of the following anatomical structures?
- Scarpa's fascia in the abdominal wall
 - Colles' fascia in the perineum
 - The urogenital diaphragm
 - The pelvic brim
- Q44. A 27-year-old female presents with secondary amenorrhea. She fails to demonstrate withdrawal bleeding following a progesterone challenge. Subsequently, a 21-day course of combined estrogen and progesterone is administered, yet the patient fails to menstruate. Further history reveals a vigorous uterine curettage following a previous miscarriage. Which of the following is the most likely diagnosis?
- Premature ovarian failure
 - Asherman syndrome
 - Sheehan syndrome
 - Polycystic Ovarian Syndrome
- Q45. A 32-year-old nulliparous woman presents with chronic pelvic pain and secondary dysmenorrhea. Ultrasonography demonstrates a 4 cm x 4 cm unilocular adnexal cyst with diffuse, low-level internal echoes (ground-glass appearance). Which of the following is the gold-standard diagnostic modality to confirm the presence of ectopic endometrial stroma and glands?
- Serum CA-125 levels
 - Transvaginal Color Doppler
 - Diagnostic laparoscopy with biopsy
 - Magnetic Resonance Imaging (MRI)
- Q46. A primigravida at term is in the second stage of labor. Clinical assessment reveals the fetal scalp at the pelvic floor, with the leading bony point at station +3. The sagittal suture is in the anteroposterior diameter of the pelvis. The obstetrician prepares to apply forceps. Which of the following is NOT a requirement for the procedure to be categorized specifically as an "outlet" forceps application?
- The fetal skull has reached the pelvic floor
 - The scalp is visible at the introitus without separating the labia
 - The fetal head is at station 0 or +1
 - The fetal head is in the occipito-anterior position
- Q47. A 22-year-old female presents with recurrent, self-limiting paroxysmal pelvic pain occurring on day 14 of a 28-day cycle. The pain is associated with a transient rise in peritoneal fluid on ultrasonography and a minor episode of spotting. This physiological event is primarily triggered by which of the following?
- Prostaglandin-mediated uterine contractions during menses
 - Follicular rupture and peritoneal irritation by follicular fluid
 - Ischemic necrosis of a subserosal leiomyoma
 - Retrograde menstruation and ectopic endometrial implantation
- Q48. A patient with a history of post-traumatic postpartum hemorrhage managed with uterine curettage presents with secondary amenorrhea and infertility. A diagnostic hysteroscopy confirms the presence of



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dense, fibrous intrauterine synechiae. The failure of the patient to exhibit withdrawal bleeding following a sequential estrogen-progesterone challenge is indicative of:

- a) Hypothalamic-adenohypophyseal axis failure
- b) Premature exhaustion of the follicular pool
- c) End-organ insensitivity due to endometrial obliteration
- d) Failure of the corpus luteum to undergo involution

Q49. A 24-year-old primigravida at 32 weeks gestation is evaluated in the antenatal clinic. Abdominal palpation reveals the uterine fundus at a level two-thirds of the distance between the umbilicus and the xiphisternum. Which of the following landmarks is most consistent with a gestational age of 36 weeks?

- a) At the level of the umbilicus
- b) One-third the distance between the umbilicus and xiphisternum
- c) At the level of the xiphisternum
- d) Midway between the symphysis pubis and umbilicus

Q50. A 26-year-old female with primary infertility is found to have an endocervical infection with an obligate intracellular bacterium. The clinician emphasizes the need for partner notification and treatment to prevent the development of hydrosalpinx and tubal factor infertility. What is the pharmacological agent of choice for this infection?

- a) Metronidazole
- b) Azithromycin
- c) Fluconazole
- d) Ceftriaxone

Q51. During the active management of the third stage of labor (AMTSL), the obstetrician observes signs of placental separation. To facilitate expulsion while minimizing the risk of uterine inversion or iatrogenic cord avulsion, a maneuver is performed involving steady traction on the umbilical cord synchronized with suprapubic counter-traction directed cephalad against the uterine body. Which of the following describes this specific procedure?

- a) Johnson maneuver
- b) Ritgen maneuver
- c) Modified Brandt-Andrews technique
- d) Crede maneuver

Q52. A 25-year-old female presents with acute pelvic pain and syncope. Clinical findings include cervical motion tenderness and a palpable adnexal mass. The patient is hemodynamically stable but exhibits a suboptimal rise in quantitative $\beta\beta$ -hCG levels over 48 hours. While imaging is suggestive, which of the following remains the gold-standard diagnostic modality for definitive visualization and potential therapeutic intervention?

- a) Transvaginal ultrasonography
- b) Diagnostic laparoscopy
- c) Magnetic Resonance Imaging
- d) Culdocentesis

Q53. A 42-year-old multigravida at 12 weeks gestation presents with hyperemesis and a fundal height significantly exceeding the gestational age. Ultrasonography reveals a "snowstorm" appearance within the endometrial cavity alongside a co-existing fetus. Histopathological analysis is expected to reveal a triploid karyotype. Which of the following is the most likely chromosomal configuration for this gestosis?



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- a) 46, XX
- b) 45, X
- c) 69, XXY
- d) 47, XX+21

Q54. A patient in labor with a twin gestation is being evaluated for the mode of delivery. Twin A is in a longitudinal cephalic presentation. Which of the following fetal configurations or chorionicities represents an absolute contraindication to a trial of vaginal delivery?

- a) Dichorionic-diamniotic twins with Twin B in breech presentation
- b) Monochorionic-diamniotic twins with Twin B in transverse lie
- c) Monochorionic-monoamniotic twins
- d) Dichorionic-diamniotic twins with Twin B in cephalic presentation

Q55. In a case of refractory postpartum hemorrhage (PPH) where conservative measures and uterotonics have failed, the surgical team decides to perform a stepwise devascularization of the uterus to avoid a hysterectomy. Following the ligation of the uterine arteries and the utero-ovarian anastomoses, which vessel should be ligated next to achieve maximal reduction in pelvic pulse pressure?

- a) Posterior division of the internal iliac artery
- b) External iliac artery
- c) Anterior division of the internal iliac artery
- d) Common iliac artery

Q56. A 23-year-old woman presents with a 3-month history of secondary amenorrhea and rapid-onset virilization, including clitoromegaly and deepening of the voice. Pelvic imaging reveals a unilateral, solid adnexal mass. Biochemical analysis confirms profound hyperandrogenemia. Which of the following is the most likely histopathological diagnosis?

- a) Sertoli-Leydig cell tumor
- b) Granulosa cell tumor
- c) Dysgerminoma
- d) Mature cystic teratoma

Q57. A 42-year-old multigravida at 11 weeks gestation presents with hyperemesis gravidarum and a fundal height significantly discordant with her documented last menstrual period. Ultrasonography demonstrates a heterogeneous endometrial mass with numerous anecdotal spaces and a co-existing non-viable fetus. Histopathological evaluation of the products of conception is most likely to reveal which of the following cytogenetic profiles?

- a) 46, XX of purely paternal origin
- b) 69, XXY resulting from diandry
- c) 45, X with cystic hygroma
- d) 47, XX, +18

Q58. A patient with a confirmed diamniotic twin gestation presents in active labor at 37 weeks. Twin A is documented in a longitudinal cephalic presentation. Which of the following specific clinical scenarios represents an absolute contraindication to a trial of vaginal delivery, necessitating an elective primary Cesarean section?

- a) Twin B in a transverse lie
- b) Monochorionic monoamniotic placentation
- c) Twin B in a frank breech presentation



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d) Dichorionic diamniotic placentation with Twin B in cephalic presentation

Q59. A 32-week pregnant female presents with rhythmic uterine contractions and a positive biochemical assay for fetal fibronectin (fFN) in the cervicovaginal secretions. Transvaginal sonography reveals a cervical length of 18 mm with "U-shaped" herniation of the fetal membranes into the endocervical canal. What is the most likely diagnosis?

- a) Cervical incompetence in the second trimester
- b) Preterm labor
- c) Pre-labor rupture of membranes (PROM)
- d) Placental abruption

Q60. In the management of life-threatening, refractory postpartum hemorrhage (PPH) where uterine atony persists despite maximal uterotonic therapy and uterine compression, the surgical team performs a stepwise devascularization. To achieve an 85% reduction in pelvic arterial pulse pressure, which of the following ligations must be performed as the final step of the devascularization sequence?

- a) Bilateral uterine artery ligation
- b) Bilateral ovarian artery ligation
- c) Anterior division of the internal iliac artery
- d) Posterior division of the internal iliac artery

Q61. A 23-year-old female presents with rapid-onset hirsutism, clitoromegaly, and secondary amenorrhea. Pelvic imaging identifies a solid, unilateral adnexal mass. Biochemical profiling demonstrates a significant elevation in serum testosterone with suppressed gonadotropins. Which of the following is the most likely pathological entity?

- a) Polycystic Ovarian Syndrome (PCOS)
- b) Sertoli-Leydig cell tumor
- c) Dysgerminoma
- d) Theca lutein cysts

Q62. A 21-year-old female presents with a primary complaint of infrequent menses. She reports that her menstrual cycles consistently occur at intervals ranging from 42 to 50 days, though the qualitative nature and volume of the flow remain within physiological limits. Which of the following clinical terms precisely categorizes this rhythmic alteration?

- a) Hypomenorrhea
- b) Oligomenorrhea
- c) Polymenorrhea
- d) Metrorrhagia

Q63. A 62-year-old postmenopausal patient presents with a solitary episode of painless vaginal spotting. Transvaginal ultrasonography (TVS) demonstrates an endometrial stripe thickness of 12 mm. A subsequent fractionated curettage and histopathological examination reveal a high density of architectural crowding with significant cellular pleomorphism and loss of polarity. What is the statistically significant risk of this specific lesion progressing to an invasive endometrioid adenocarcinoma?

- a) 1%
- b) 8%
- c) 29%
- d) 54%



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- Q64. A 35-year-old female undergoes a cytological screening of the transformation zone. The laboratory report indicates the presence of "koilocytic atypia" and "perinuclear haloing," suggestive of a low-grade squamous intraepithelial lesion (LSIL). What is the primary clinical objective of this specific diagnostic modality?
- Secondary prevention of cervical malignancy
 - Definitive confirmation of *Trichomonas vaginalis* colonization
 - Primary screening for high-grade serous ovarian carcinoma
 - Histopathological grading of endometrial hyperplasia
- Q65. A couple is being evaluated for primary infertility. A Sims-Huhner test (post-coital test) is performed during the mid-cycle surge of luteinizing hormone. Which of the following etiologies of subfertility is this specific investigative modality incapable of assessing?
- Dysmucorrhea (hostile cervical mucus)
 - Antisperm antibody-mediated immobilization
 - Bilateral salpingeal cornual occlusion
 - Asthenozoospermia within the cervical reservoir
- Q66. A 40-year-old nulliparous patient presents with progressive pelvic heaviness. Laparotomy reveals an adnexal cyst containing sebaceous-like debris and organized haemosiderin, with histopathology demonstrating ectopic endometrial glands and specialized stroma. Which of the following statements regarding this pathology is fundamentally incorrect?
- It represents a benign sequela of retrograde menstruation
 - The internal lining typically consists of pseudostratified columnar epithelium
 - The lesion's growth is independent of ovarian steroidogenesis
 - It is a common cause of acquired subfertility and pelvic adhesions
- Q67. A 67-year-old postmenopausal patient presents with a Stage III pelvic organ prolapse. Following the manual repositioning of the uterus, an effortless herniation of the posterior vaginal wall is observed, demonstrating distinct bowel peristalsis. The patient is sexually inactive and seeks an obliterative surgical solution. Which procedure is specifically designed to address this posterior pulsion diverticulum of the Pouch of Douglas?
- Anterior colporrhaphy
 - Moschcowitz procedure
 - Manchester operation
 - Shirodkar's cerclage
- Q68. A 14-week primigravida presents with intractable hyperemesis, thyrotoxicosis-like symptoms, and a uterine fundus palpated at the level of the umbilicus. Sonographic evaluation reveals a "cluster of grapes" appearance within the endometrial cavity with a total absence of fetal adnexa or a gestational sac. Quantitative $\beta\beta$ -hCG is disproportionately elevated. Following hemodynamic stabilization, which of the following represents the definitive primary management to mitigate the risk of pulmonary trophoblastic embolization and malignant transformation?
- Prophylactic methotrexate therapy
 - Total hysterectomy with conservation of adnexa
 - Suction curettage and sharp curettage under ultrasonographic guidance
 - Expectant management with serial $\beta\beta$ -hCG monitoring



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- Q69. A 28-year-old female experiences refractory atonic postpartum hemorrhage (PPH) unresponsive to high-dose oxytocin and carboprost. The obstetrician utilizes a hydrostatic tamponade device to achieve hemostasis. Which of the following is the physiologically appropriate volume of sterile crystalloid required to achieve the "tamponade test" effectively?
- 50–150 mL
 - 300–500 mL
 - 800–1000 mL
 - 1200–1500 mL
- Q70. A patient presents with a malodorous, homogeneous, leucorrheal discharge. Clinical assessment reveals a vaginal pH of 5.5 and the release of volatile amines upon the addition of 10% potassium hydroxide. Microscopy demonstrates squamous epithelial cells with a stippled, "shaggy" border due to the adherence of coccobacilli. These pathognomonic cells are known as:
- Tzanck cells
 - Clue cells
 - Koilocytes
 - Parabasal cells
- Q71. A 22-year-old female with a consistent 37-day menstrual cycle seeks preconception counseling. Assuming a stable and physiological luteal phase, on which day of her cycle is the LH surge and subsequent follicular rupture most likely to occur?
- Day 14
 - Day 18
 - Day 21
 - Day 23
- Q72. A multigravida with a history of catastrophic peripartum hemorrhage requiring massive transfusion presents with secondary amenorrhea and an inability to initiate lactation. Physical examination reveals an absence of pubic and axillary hair. Which of the following is the primary pathophysiological mechanism?
- Autoimmune destruction of the adrenal cortex
 - Ischemic necrosis of the adenohypophysis
 - Fibrotic obliteration of the endometrial cavity
 - Hyperprolactinemia-induced gonadotropin suppression
- Q73. A 16-year-old female with primary amenorrhea and short stature is found to have a 45,X karyotype. Which of the following biochemical profiles is most consistent with her condition of gonadal dysgenesis?
- High Estrogen, Low FSH
 - High FSH, High LH, Low Estrogen
 - Low FSH, Low LH, Low Estrogen
 - Normal FSH, High Testosterone
- Q74. A 13-year-old female presents with primary amenorrhea and progressively worsening, rhythmic monthly pelvic pain. Physical examination reveals a distended, translucent, violaceous membrane at the introitus. Which of the following is the most likely sequela if this condition remains untreated?
- Hematocolpos and Hematometra
 - Ovarian agenesis
 - Testicular feminization
 - Endometrial atrophy



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- Q75. A 13-year-old girl presents to the emergency department with acute lower abdominal pain. She gives a history of recurrent cyclical abdominal pain for the past 6 months but has not yet attained menarche. On examination, there is a tense, bluish bulge noted at the level of the hymen. Abdominal examination suggests a pelvic mass due to retained menstrual blood. What is the most probable diagnosis?
- Mayer-Rokitansky-Küster-Hauser (MRKH) syndrome
 - Testicular feminization syndrome
 - Imperforate hymen
 - Asherman's syndrome
- Q76. A 38-year-old multiparous female presents with a constellation of symptoms including periorbital myxedema, bradypsychia, and a notable prolongation of the relaxation phase of the Achilles tendon reflex. Laboratory analysis reveals a serum TSH of 25 mU/L and a diminished free thyroxine (T_4) index. To delineate the autoimmune substrate of this primary endocrinopathy, which of the following humoral markers is the most specific and sensitive for the suspected underlying pathology?
- Anti-sodium-iodide symporter (NIS) antibodies
 - Anti-thyroid peroxidase (TPO) antibodies
 - Thyrotropin receptor-stimulating immunoglobulins (TSI)
 - Anti-thyroglobulin (Tg) antibodies
- Q77. A 32-year-old male is evaluated for neuroglycopenic symptoms that resolve promptly upon the administration of simple carbohydrates. During a symptomatic episode, biochemical analysis reveals a plasma glucose of 40 mg/dL, an inappropriately elevated serum insulin level, and a suppressed C-peptide concentration. Which of the following clinical scenarios is most consistent with this metabolic profile?
- Autonomous hypersecretion from a pancreatic neuroendocrine tumor
 - Surreptitious administration of exogenous regular insulin
 - Factitious ingestion of second-generation sulfonylureas
 - Non-islet cell tumor-induced hypoglycemia (NICTH)
- Q78. A 45-year-old male with a history of poorly controlled metabolic syndrome presents with sudden-onset, large-amplitude, violent, flinging movements of the right-sided proximal musculature. The movements persist during wakefulness and cease during sleep. Disruption of the indirect pathway of the basal ganglia is suspected. Which of the following neuroanatomical structures is the most likely site of the vascular insult?
- Pars compacta of the substantia nigra
 - Contralateral subthalamic nucleus
 - Ipsilateral globus pallidus externa
 - Putamen and caudate nucleus
- Q79. A 28-year-old male presents with chronic inflammatory axial arthropathy and restricted chest expansion. He also reports symptoms consistent with acute anterior uveitis. Laboratory evaluation reveals a seronegative profile for rheumatoid factor and antinuclear antibodies. If this patient's condition progresses to its end-stage, which of the following pleuropulmonary complications is most characteristically associated with this specific spondyloarthropathy?
- Necrobiotic lung nodules
 - Apical fibrobullous disease
 - Diffuse panbronchiolitis



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d) Caplan syndrome

Q80. A 10-year-old male presents with an episode of macroscopic hematuria that manifested 24 hours after the onset of an acute pharyngitis. The patient has no peripheral edema and a normal blood pressure. Laboratory analysis shows a serum C3 level within the physiological range. Which of the following is the most likely histopathological finding on renal biopsy?

- a) Diffuse endocapillary proliferation with "humps" on electron microscopy
- b) Linear IgG deposition along the glomerular basement membrane
- c) Mesangial hypercellularity with prominent IgA immunofluorescence
- d) Effacement of podocyte foot processes on electron microscopy

Q81. A 40-year-old female presents with a 5-month history of progressive bradypsychia, cold intolerance, and a change in vocal resonance characterized by low-pitched hoarseness. Physical examination is significant for non-pitting edema of the pretibial and facial regions and a characteristic "hung-up" sensation during the assessment of the calcaneal tendon reflex. Laboratory evaluation confirms a state of hypometabolism. Which of the following biochemical profiles is most consistent with the primary form of this endocrinopathy?

- a) Diminished TSH, Diminished Free T4, Normal T3
- b) Elevated TSH, Diminished Free T4, Diminished Free T3
- c) Elevated TSH, Elevated Free T4, Diminished Free T3
- d) Diminished TSH, Elevated Free T4, Elevated Free T3

Q82. A 50-year-old male presents with constitutional symptoms and abdominal distension. Radiographic evaluation identifies a neoplastic process at the duodenojejunal junction with direct transmural infiltration into the retroperitoneal space, involving the renal parenchyma. Systemic staging via PET-CT fails to identify any lymphadenopathy or distant visceral metastases. Histopathology confirms a non-Hodgkin lymphoma. According to the Lugano modification of the Ann Arbor staging system, what is the most accurate stage for this presentation?

- a) Stage I E
- b) Stage II E
- c) Stage III E
- d) Stage IV

Q83. A 55-year-old male with a history of chronic ethanol consumption exhibits clinical markers of portal hypertension. The underlying hepatic architecture is characterized by diffuse parenchymal transformation into regenerative nodules. Which of the following mesenchymal cells, located within the perisinusoidal space of Disse, undergoes myofibroblastic transdifferentiation to serve as the primary effector of this fibrotic process?

- a) Kupffer cells
- b) Ito cells
- c) Cholangiocytes
- d) Pit cells

Q84. A patient with a documented pulmonary malignancy presents with truncal obesity, violaceous striae, and significant hyperpigmentation. Laboratory analysis reveals refractory hypokalemic metabolic alkalosis and markedly elevated plasma ACTH. Which of the following features most accurately distinguishes this "ectopic" presentation from a pituitary-driven Cushing's disease?

- a) Suppression of cortisol in response to high-dose dexamethasone



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- b) Hypokalemia due to mineralocorticoid crossover effects
- c) Regression of symptoms following bilateral adrenalectomy
- d) Presence of bitemporal hemianopia

Q85. A 45-year-old female presents with a pleural effusion. Thoracentesis demonstrates a pleural fluid-to-serum protein ratio of 0.75 and a pleural fluid LDH-to-serum LDH ratio of 0.8. The adenosine deaminase (ADA) activity in the aspirate is 100 U/L. Which of the following pathophysiological mechanisms is primarily responsible for the elevation of this specific enzyme?

- a) Increased capillary hydrostatic pressure
- b) Activation of T-lymphocytes by Mycobacterium tuberculosis antigens
- c) Decreased plasma oncotic pressure due to hypoalbuminemia
- d) Malignant infiltration of the pleura causing lymphatic obstruction

Q86. An 18-year-old male presents with sudden-onset anasarca following a prodromal viral syndrome. Urinalysis reveals 4+ proteinuria with a "bland" sediment, and light microscopy of the renal biopsy demonstrates histologically unremarkable glomeruli. Which of the following ultrastructural changes would be expected on electron microscopy?

- a) Subepithelial "humps" of immune complexes
- b) Diffuse effacement of podocyte foot processes
- c) Splitting of the glomerular basement membrane
- d) Mesangial interposition and "tram-track" appearance

Q87. A patient being treated for rheumatoid arthritis develops a normocytic anemia and persistent diarrhea. The clinician suspects toxicity from a disease-modifying antirheumatic drug (DMARD) that undergoes bacterial cleavage in the colon into sulfapyridine and 5-aminosalicylic acid. Which of the following is the most likely culprit?

- a) Methotrexate
- b) Sulfasalazine
- c) Leflunomide
- d) Etanercept

Q88. A 28-year-old male with a history of intravenous drug use and clinical progression to Stage 3 HIV infection presents with anasarca and refractory hypertension. Renal biopsy demonstrates a "collapsing" pattern of glomerular capillary loops with global podocyte hypertrophy. Additionally, the interstitium shows marked microcystic tubular dilatation containing eosinophilic, proteinaceous casts. Which of the following is the most likely pathological diagnosis?

- a) Diffuse proliferative glomerulonephritis
- b) Focal segmental glomerulosclerosis
- c) Membranous nephropathy
- d) Amyloidosis

Q89. A 70-year-old male with a history of chronic obstructive pulmonary disease (COPD) is being evaluated for long-term prognosis. Spirometric analysis is performed to determine the degree of airflow limitation. According to the Global Initiative for Chronic Obstructive Lung Disease (GOLD) guidelines, which of the following parameters is the most robust independent predictor of all-cause mortality and frequency of exacerbations?

- a) Total Lung Capacity (TLC)
- b) Post-bronchodilator FEV_1 % predicted



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- c) Inspiratory Capacity to Total Lung Capacity (*IC/TLC*) ratio
- d) Peak Expiratory Flow (PEF)

- Q90. A 60-year-old male with end-stage renal disease (ESRD) on long-term renal replacement therapy develops macroscopic hematuria. Computed tomography identifies a circumscribed cortical mass. Histological examination reveals a papillary growth pattern with foamy macrophages within the fibrovascular cores and numerous psammoma bodies. Cytogenetic analysis is most likely to reveal which of the following?
- a) Translocation $t(X; 17)t(X; 17)$
 - b) Deletion of 3p (*VHL* gene)
 - c) Trisomy 7 and 17
 - d) *MET* proto-oncogene deletion
- Q91. A 45-year-old patient presents with a unilateral pleural effusion. Thoracentesis yields a fluid with a protein concentration of 4.0 g/dL and a glucose concentration of 35 mg/dL. The Adenosine Deaminase (ADA) level is measured at 110 U/L. Which of the following cells is the primary source of the elevated enzyme found in this aspirate?
- a) Neutrophilic granulocytes
 - b) Mesothelial cells
 - c) Activated T-lymphocytes
 - d) Eosinophilic leukocytes
- Q92. An 18-year-old male presents with sudden-onset periorbital edema and nephrotic-range proteinuria (4.5 g/day) following an upper respiratory tract infection. Light microscopy of the renal biopsy reveals no significant glomerular abnormalities, and immunofluorescence is negative for immune deposits. What is the most likely mechanism of injury in this patient?
- a) Type III hypersensitivity-mediated immune complex deposition
 - b) Cytokine-induced effacement of visceral epithelial cell foot processes
 - c) Deposition of misfolded amyloidogenic light chains
 - d) Autoantibody-mediated destruction of the glomerular basement membrane
- Q93. A 42-year-old female with chronic symmetric polyarthritis presents with persistent diarrhea and a macrocytic anemia. The patient is currently receiving a disease-modifying antirheumatic drug (DMARD) that is an azo-bonded compound of a salicylate and a sulfonamide. Which of the following mechanisms best explains her hematological finding?
- a) Direct inhibition of dihydrofolate reductase
 - b) Interference with the intestinal absorption of folate
 - c) Myelosuppression due to Janus kinase inhibition
 - d) Sequestration of iron within the reticuloendothelial system
- Q94. A 30-year-old male presents with exertional presyncope. Physical examination reveals a low-amplitude carotid upstroke with a delayed peak and a palpable systolic thrill in the second right intercostal space. Auscultation identifies a grade IV/VI harsh crescendo-decrescendo murmur that persists despite a Valsalva maneuver and radiates cephalad toward the carotid bifurcations. The second heart sound (S_2) is markedly attenuated. Which of the following hemodynamic consequences is most likely associated with the chronic progression of this valvular pathology?
- a) Eccentric hypertrophy with increased ventricular compliance
 - b) Concentric hypertrophy with impaired diastolic relaxation



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- c) Paradoxical splitting of the second heart sound due to early pulmonic closure
- d) Dynamic outflow obstruction that increases with reduced preload

Q95. A 68-year-old patient is evaluated eight weeks post-ST-elevation myocardial infarction (STEMI) involving the left anterior descending artery. The patient demonstrates a persistent ST-segment elevation on ECG and a localized paradoxical precordial impulse during ventricular systole. Echocardiography confirms a dyskinetic, thinned myocardial segment. Which of the following represents the most frequent clinical sequela of this structural abnormality?

- a) Acute cardiac tamponade due to transmural rupture
- b) Progressive congestive heart failure
- c) Systemic embolization from a mural thrombus
- d) Development of a secondary VSD in the muscular septum

Q96. A 68-year-old male with a history of ischemic cardiomyopathy and a reduced ejection fraction (30%) is currently optimized on a beta-adrenoceptor antagonist and an aldosterone antagonist. To further attenuate the risk of cardiovascular mortality and reduce the rate of hospitalization for heart failure, which of the following mechanisms of action should be targeted next in his foundational regimen?

- a) Inhibition of the I_f -If current in the sinoatrial node
- b) Inhibition of the sodium-glucose cotransporter in the proximal convoluted tubule
- c) Positive inotropy via inhibition of the Na^+/K^+ -ATPase pump
- d) Antagonism of L-type calcium channels in the myocardium

Q97. A 28-year-old female presents with paroxysmal nocturnal dyspnea and cough. Pulmonary function testing demonstrates a reduction in FEV_1 that exceeds 12% and 200 mL following the administration of a β_2 -agonist. Which of the following histopathological features is most characteristic of the underlying chronic inflammatory process in this patient's airways?

- a) Destructive enlargement of airspaces distal to the terminal bronchiole
- b) Subepithelial collagen deposition and Curschmann spirals
- c) Permanent abnormal dilation of the bronchi with signet-ring signs
- d) Pan-acinar alveolar wall destruction with alpha-1 antitrypsin deficiency

Q98. A 68-year-old male presents with a sudden-onset expressive aphasia and a right-sided hemiparesis that is more pronounced in the face and upper extremity than in the lower extremity. A non-contrast CT of the head is negative for hemorrhage. Which of the following vascular structures is the most likely site of the thromboembolic occlusion?

- a) Right Anterior Cerebral Artery (ACA)
- b) Left Middle Cerebral Artery (MCA) - Superior Division
- c) Left Posterior Cerebral Artery (PCA)
- d) Basilar Artery - Tip

Q99. A 45-year-old male presents with acute, sharp precordial pain that is exacerbated by the supine position and deep inspiration, but significantly attenuated when leaning forward. Electrocardiography reveals diffuse, concave-upward ST-segment elevation across multiple non-contiguous leads. Which of the following additional ECG findings would most strongly support the suspected diagnosis?

- a) Pathological Q-waves in the inferior leads
- b) PR-segment depression in lead II and PR-segment elevation in aVR
- c) Convex-downward ST-segment depression in the lateral leads
- d) New-onset left bundle branch block



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- Q100. A 62-year-old male with a 40 pack-year tobacco history presents with chronic productive cough and progressive exertional dyspnea. Physical examination reveals a hyper-resonant percussion note, distant heart sounds, and an increased anteroposterior thoracic diameter. Spirometric evaluation demonstrates a post-bronchodilator FEV_1/FVC ratio of 0.58. According to the Global Initiative for Chronic Obstructive Lung Disease (GOLD) pharmacological management algorithms, which of the following is the most appropriate first-line maintenance intervention to reduce the frequency of acute exacerbations?
- Regular administration of a short-acting muscarinic antagonist (SAMA)
 - Inhaled corticosteroid (ICS) monotherapy
 - Long-acting muscarinic antagonist (LAMA) or LABA/LAMA combination
 - Sustained-release methylxanthine derivatives
- Q101. A 64-year-old male presents with acute-onset diaphoresis and substernal chest pressure. Electrocardiography (ECG) demonstrates 2 mm ST-segment elevation in leads II, III, and aVF, with associated ST-segment depression in leads I and aVL. Following stabilization, the patient develops a new-onset bradyarrhythmia. Which of the following anatomical structures is most likely compromised by the primary vascular occlusion?
- Atrioventricular (AV) node via the Right Coronary Artery (RCA)
 - Bundle of His via the Septal Perforators of the LAD
 - Posterior fascicle via the Left Circumflex Artery (LCX)
 - Sinoatrial (SA) node via the Diagonal branch
- Q102. A 32-year-old female presents with a history of transient visual obscurations and paroxysmal motor weakness that exhibits "dissemination in space and time." Magnetic Resonance Imaging (MRI) of the neuraxis identifies multiple hyperintense T2/FLAIR lesions oriented perpendicular to the ventricular surface. Which of the following cerebrospinal fluid (CSF) findings would most strongly corroborate the suspected autoimmune etiology?
- Albuminocytologic dissociation
 - Pleocytosis with decreased glucose levels
 - Presence of oligoclonal IgG bands not found in serum
 - Xanthochromia with elevated opening pressure
- Q103. A 72-year-old male exhibits an insidious decline in executive function and episodic memory. Neuroimaging reveals global cortical volume loss with disproportionate atrophy of the entorhinal cortex and hippocampus. Which of the following biochemical alterations is the primary driver of the intraneuronal filamentous aggregates associated with this condition?
- Extracellular deposition of misfolded Alpha-synuclein
 - Hyperphosphorylation of the microtubule-associated Tau protein
 - Expansion of Cytosine-Guanine-Guanine (CGG) repeats
 - Proteolytic cleavage of Amyloid Precursor Protein (APP) into Beta-amyloid
- Q104. A 45-year-old male presents with acute, sharp, retrosternal pain that is relieved by sitting upright and leaning forward. Auscultation reveals a high-pitched, scratchy, triphasic sound at the left lower sternal border. Electrocardiography demonstrates diffuse, concave-upward ST-segment elevation. Which of the following ECG features is most specific for differentiating this condition from an early repolarization variant or an acute myocardial infarction?
- Reciprocal ST-segment depression in lead aVL
 - Presence of prominent J-waves in the precordial leads



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- c) PR-segment depression in lead II with reciprocal PR-elevation in aVR
- d) Convex-downward ST-segment morphology

Q105. A 32-year-old female presents with a 12-month history of paroxysmal neurologic dysfunction, including an episode of painful monocular vision loss and a subsequent bout of truncal ataxia. Physical examination reveals an afferent pupillary defect and internuclear ophthalmoplegia. Magnetic Resonance Imaging (MRI) of the brain demonstrates several T2-hyperintense foci situated perpendicular to the callososeptal interface. Which of the following immunopathological mechanisms is primarily responsible for the neuroanatomical lesions observed in this patient?

- a) Complement-mediated destruction of the postsynaptic acetylcholine receptors
- b) T-cell mediated demyelination of the central nervous system axons
- c) Molecular mimicry targeting the gangliosides of the peripheral nerves
- d) Degeneration of the corticospinal tracts and anterior horn cells

Q106. A 10-month-old infant is evaluated during a surveillance visit. The infant demonstrates the ability to maintain an upright seated position without pelvic support and exhibits a mature palmar grasp with successful interdigital transfer of a cube. During the encounter, the infant orients toward the clinician upon auditory stimulation of their name. Physical examination reveals an inability to achieve a vertical posture via pulling on external supports or maintaining a static stance with assistance. The birth history was unremarkable at 39 weeks gestation. What is the most appropriate interpretation of these findings?

- a) Normal neurodevelopmental trajectory
- b) Isolated gross motor dyspraxia
- c) Upper motor neuron dysfunction
- d) Global developmental delay

Q107. A 26-month-old toddler presents with a primary complaint of expressive communication deficits. The child possesses a functional lexicon of approximately three distinct words and exhibits an inability to generate telegraphic speech or join words into binary phrases. Receptive language is intact, as evidenced by the successful execution of unistep commands. Gross motor examination reveals the ability to ascend stairs with a marking-time gait and running with a narrow base of support. Social reciprocal play is preserved. What is the most likely diagnostic focal point?

- a) Socio-emotional deprivation
- b) Pervasive developmental disorder
- c) Isolated expressive language impairment
- d) Intellectual disability

Q108. A 6-year-old male presents with a 5-day history of progressive anasarca, specifically periorbital and pretibial edema. He denies constitutional symptoms, gross hematuria, or a decrease in urinary frequency. Physical examination reveals a blood pressure of 95/60 mmHg. Urinalysis demonstrates 4+ proteinuria and an absence of erythrocyte casts. Laboratory studies confirm a serum albumin of 1.8 g/dL and total cholesterol of 320 mg/dL. Serum creatinine and C3 complement levels are within reference ranges. What is the most probable histopathological finding on electron microscopy?

- a) Subepithelial humps and endocapillary proliferation
- b) Diffuse effacement of podocyte foot processes
- c) Mesangial IgA deposition
- d) Splitting of the glomerular basement membrane



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- Q109. A 30-week-gestation neonate develops acute respiratory insufficiency minutes after delivery. Clinical signs include paradoxical chest wall movement, expiratory phonation (grunting), and significant alar flaring. Despite high-flow oxygen, cyanosis persists. A chest radiograph reveals diffuse, fine reticulogranular opacities and prominent air-filled bronchi extending beyond the cardiac silhouette. What is the primary biochemical defect underlying this condition?
- Inactivation of surfactant by meconium salts
 - Delayed clearance of fetal lung fluid
 - Deficiency of dipalmitoylphosphatidylcholine
 - Congenital deficiency of Alpha-1 antitrypsin
- Q110. A 5-year-old male is brought to the pediatric clinic due to a 4-day history of escalating anasarca, most prominent in the periorbital and pretibial regions. The parents deny any antecedent episodes of macroscopic hematuria, pharyngitis, or pyoderma. Clinical examination reveals a normotensive child with significant pitting edema and shifting dullness on abdominal percussion, suggesting ascites. Semiquantitative urinalysis demonstrates a protein excretion level of 4+, while microscopic sediment analysis is negative for dysmorphic erythrocytes or cellular casts. Serum biochemistry reveals a serum albumin of 1.9 g/dL and a marked elevation in low-density lipoprotein (LDL) fractions. Serum C3 and C4 complement levels are within the reference range, and the antistreptolysin O (ASO) titer is non-reactive. Which of the following best describes the pathophysiological basis of this patient's condition?
- Immune-complex mediated activation of the classical complement pathway
 - Cytokine-induced neutralization of the glomerular polyanionic charge barrier
 - Genetic mutation in the NPHS1 gene encoding the nephrin protein
 - Deposition of organized microtubular structures within the mesangium
- Q111. A neonate born at 39 weeks gestation via uncomplicated spontaneous vaginal delivery is noted to have significant dermal icterus extending to the mid-torso at 18 hours of life. The maternal blood group is O-positive, and the neonatal blood group is A-positive. Serum biochemistry reveals a total bilirubin of 14 mg/dL with a direct fraction of 0.5 mg/dL. Reticulocyte count is 9%, and a peripheral blood smear demonstrates numerous microspherocytes. What is the most likely immunological finding in this patient?
- Positive Direct Antiglobulin Test (DAT)
 - Deficiency of Uridine diphosphate-glucuronosyltransferase
 - Presence of lipoprotein X in serum
 - Inactivation of the UGT1A1 promoter region
- Q112. A 2-year-old male presents with a 72-hour history of pyrexia, lethargy, and respiratory distress. Physical examination reveals a toxic-appearing child with a respiratory rate of 55 breaths/minute and use of accessory muscles. Thoracic auscultation is significant for egophony and increased vocal fremitus localized to the right infrascapular region. Percussion over the same area yields a stony dull note. A posteroanterior chest radiograph demonstrates a dense, homogeneous triangular opacity with visible radiolucent branching structures within the consolidation. What is the most likely causative pathogen?
- Respiratory Syncytial Virus
 - Streptococcus pneumoniae
 - Mycoplasma pneumoniae
 - Chlamydia trachomatis
- Q113. A 7-month-old infant is evaluated for acute-onset expiratory wheezing and tachypnea following a 2-day prodrome of coryza and low-grade fever. Clinical examination reveals a pulse oximetry of 89% on room air, subcostal recessions, and diffuse fine end-expiratory crackles on auscultation. A chest radiograph shows



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depression of the diaphragmatic domes, an increase in the retrosternal clear space, and attenuation of peripheral vascular markings. What is the primary anatomical site of pathology?

- a) Alveolar-capillary interface
- b) Terminal and respiratory bronchioles
- c) Large cartilaginous airways
- d) Pleural space

Q114. An 11-year-old male presents with persistent edema and proteinuria. He was previously diagnosed with nephrotic syndrome and initiated on a standard 6-week course of high-dose corticosteroids, but he failed to achieve remission. A renal biopsy is performed, and light microscopy reveals segmental collapse of the glomerular tuft with hyaline deposition and adhesions to the Bowman capsule in approximately 30% of the sampled glomeruli. Immunofluorescence shows coarse granular deposits of IgM and C3 in the affected segments. What is the most likely diagnosis?

- a) Minimal Change Disease
- b) Focal Segmental Glomerulosclerosis (FSGS)
- c) Membranous Nephropathy
- d) Diabetic Nephropathy

Q115. An 8-year-old child presents with a sudden onset of "tea-colored" urine and facial edema. The clinical history is significant for a self-limited febrile pharyngitis 18 days prior. Physical examination reveals a blood pressure at the 98th percentile for age and height. Urinalysis is positive for dysmorphic erythrocytes and erythrocyte casts. Serum analysis reveals a significantly depressed C3 level with a normal C4 level. What is the most likely immunopathogenesis?

- a) Type II hypersensitivity against the glomerular basement membrane
- b) Alternative complement pathway activation via immune complex deposition
- c) IgA-mediated mesangial proliferation
- d) T-cell mediated podocytopathy

Q116. A 96-hour-old full-term neonate is evaluated for progressive dermal icterus. Physical examination reveals that the yellow discoloration is evident on the forehead, thorax, and has reached the level of the umbilical perimeters, but the popliteal fossae and distal extremities remain unaffected. The neonate is vigorous with normal primitive reflexes and no evidence of hepatosplenomegaly. According to the cephalocaudal progression patterns described by Kramer, what is the most likely range of the total serum bilirubin (TSB) in this infant?

- a) 4.0 – 6.0 mg/dL
- b) 6.1 – 8.5 mg/dL
- c) 9.0 – 11.5 mg/dL
- d) 14.0 – 16.5 mg/dL

Q117. A 7-year-old child presents with a subacute history of morning emesis, truncal titubation, and dysmetria. Neurological examination reveals bilateral papilledema and a broad-based gait. Magnetic resonance imaging (MRI) of the neuraxis demonstrates a solid, contrast-enhancing mass arising from the roof of the fourth ventricle and occupying the cerebellar vermis. Histopathological analysis shows densely packed small round blue cells forming Homer Wright rosettes. What is the most likely diagnosis?

- a) Pilocytic Astrocytoma
- b) Medulloblastoma
- c) Supratentorial Ependymoma
- d) Craniopharyngioma



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- Q118. A 9-year-old patient with a history of minimal change disease (MCD) presents with a third recurrence of nephrotic-range proteinuria within a single calendar year. Each prior episode achieved complete remission with standard-dose prednisolone, but the current relapse occurred while the steroid dose was being tapered below 0.5 mg/kg. The patient exhibits early signs of iatrogenic Cushing syndrome. According to current pediatric nephrology guidelines, which immunomodulatory agent is the most appropriate initial steroid-sparing intervention?
- Levamisole
 - Rituximab
 - Mycophenolate Mofetil (MMF)
 - Cyclosporine A
- Q119. A 4-year-old child is brought to the emergency department in acute respiratory distress. The patient is febrile, tachycardic, and assumes a posture of cervical hyperextension with the mandible thrust forward. Significant sialorrhea is noted, and the child's voice has a "thick" quality, though no spontaneous cough is present. While preparing for intervention, what is the most critical management principle to observe?
- Immediate visualization of the oropharynx using a tongue depressor
 - Procurement of a lateral soft-tissue neck radiograph to evaluate the "thumb sign"
 - Avoidance of any distressing maneuvers until a definitive airway is established in a controlled environment
 - Administration of nebulized racemic epinephrine to reduce supraglottic edema
- Q120. A 2-year-old female presents with refractory microcytic anemia, abdominal distension, and significant myofibrillar wasting of the gluteal region. The stool is described as voluminous, pale, and foul-smelling. An endoscopic duodenal biopsy is performed, revealing total obliteration of the valvulae conniventes, an increased Mitotic Index in the crypts of Lieberkühn, and a Marsh Grade 3c classification. What is the most likely underlying immunopathogenesis?
- IgE-mediated hypersensitivity to bovine milk proteins
 - T-cell mediated sensitivity to the gliadin fraction of prolamins
 - Autoimmune destruction of the H⁺/K⁺ ATPase pump in parietal cells
 - Congenital absence of the SLC5A1 glucose-galactose cotransporter
- Q121. A 7-year-old child from an urban settlement presents with progressive ambulatory dysfunction and genu varum. Clinical examination reveals palpable enlargement of the costochondral junctions and a distinct prominence of the accessory ossification center of the medial malleolus, creating a "step-off" appearance. Radiographic assessment of the distal radius demonstrates a loss of the normal provisional zone of calcification, with associated splaying and concave indentation of the metaphysis. Which of the following biochemical profiles is most consistent with the early stage of this pathology?
- Hypercalcemia, hypophosphatemia, and suppressed Alkaline Phosphatase
 - Hypocalcemia, hypophosphatemia, and elevated serum Parathyroid Hormone
 - Hyperphosphatemia, hypocalcemia, and low 1,25-dihydroxyvitamin D
 - Normal serum calcium, hyperuricemia, and elevated Vitamin C levels
- Q122. A 15-month-old child is evaluated for an abnormal cranial configuration characterized by a prominent midline frontal ridge and a keel-shaped forehead. Physical examination is significant for a decreased inter-pupillary distance (hypotelorism). Development is otherwise within normal parameters for age. Which of the following embryological events is responsible for this specific phenotype?
- Premature synostosis of the sagittal suture
 - Isolated premature ossification of the metopic suture



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- c) Unilateral fusion of the hemicoronal suture
- d) Bilateral synostosis of the lambdoid sutures

Q123. A military recruit undergoing basic combat training presents with an insidious onset of focal pain over the middle third of the anterior tibial diaphysis. The discomfort is strictly related to weight-bearing activities and is exquisitely localized to a 2-cm area of point tenderness. Plain film radiography is initially unremarkable. What is the most likely diagnosis, and what is the optimal imaging modality for early confirmation?

- a) Medial Tibial Stress Syndrome; Clinical diagnosis
- b) Tibial Stress Fracture; Magnetic Resonance Imaging (MRI)
- c) Chronic Exertional Compartment Syndrome; Intracompartmental pressure manometry
- d) Osteoid Osteoma; Computed Tomography (CT)

Q124. An 80-year-old female presents with a "dinner-fork" deformity following a low-energy fall on an outstretched hand. Radiographs confirm a distal radius fracture with dorsal angulation and comminution. Following closed reduction and immobilization in a below-elbow cast, the patient is counseled on the importance of immediate range-of-motion exercises. Which of the following is the most frequent long-term morbidity associated with this specific injury?

- a) Rupture of the Extensor Pollicis Longus (EPL) tendon
- b) Complex Regional Pain Syndrome (CRPS) Type I
- c) Digital stiffness and restricted grip strength
- d) Osteonecrosis of the distal radial epiphysis

Q125. A 45-year-old female presents with diffuse, dull-aching skeletal pain and proximal myopathy, resulting in a "waddling" gait. Radiographic survey of the pelvis demonstrates bilateral, symmetric, transverse radiolucent bands perpendicular to the cortex of the inferior pubic rami. Biochemical analysis reveals an elevated serum Alkaline Phosphatase and low 25-hydroxyvitamin D. What is the most likely diagnosis?

- a) Osteitis Fibrosa Cystica
- b) Osteoporosis
- c) Osteomalacia
- d) Paget's Disease of Bone

Q126. A 45-year-old male presents with a persistent, pruritic eruption characterized by violaceous, polygonal, flat-topped papules localized to the flexor surfaces of the wrists and the lumbar region. On close inspection with dermoscopy, a delicate network of white reticular lines is visible on the surface of the lesions. The patient also complains of painful erosions on the buccal mucosa. A punch biopsy is performed, revealing a band-like lymphohistiocytic infiltrate at the dermo-epidermal junction, liquefactive degeneration of the basal layer, and the presence of eosinophilic, shrunken keratinocytes in the lower epidermis. What is the most likely diagnosis?

- a) Psoriasis Vulgaris
- b) Lichen Planus
- c) Lupus Erythematosus
- d) Pityriasis Rosea

Q127. A 60-year-old female presents with the sudden onset of large, tense, subepidermal bullae on an erythematous base, primarily involving the intertriginous areas and the lower abdomen. The patient reports intense pruritus but denies significant mucosal involvement. Nikolsky sign is negative. Direct



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immunofluorescence (DIF) of perilesional skin demonstrates a continuous, linear deposition of IgG and C3 along the basement membrane zone. Enzyme-linked immunosorbent assay (ELISA) is positive for antibodies against a 230-kDa hemidesmosomal protein. What is the primary target of the autoantibodies in this condition?

- a) Desmoglein 3
- b) Desmoglein 1
- c) BPAG1 (BP230)
- d) Type VII Collagen

Q128. A 35-year-old male presents with symmetric, targetoid lesions consisting of a central dusky zone, a surrounding pale edematous ring, and an erythematous outer halo. The lesions are concentrated on the extensor surfaces of the distal extremities. The patient reports a recurrent history of labial herpes simplex infections, with the current eruption appearing ten days after the last cold sore. There is no systemic involvement or mucosal sloughing. What is the most likely diagnosis?

- a) Erythema Multiforme Minor
- b) Stevens-Johnson Syndrome
- c) Toxic Epidermal Necrolysis
- d) Bullous Fixed Drug Eruption

Q129. A 28-year-old female presents with a "butterfly" malar rash that spares the nasolabial folds, along with photosensitivity and non-scarring alopecia. Laboratory evaluation reveals a positive Antinuclear Antibody (ANA) with a homogeneous pattern and a high titer of Anti-double-stranded DNA (dsDNA) antibodies. Histopathological examination of the skin lesions shows vacuolar interface dermatitis, basement membrane thickening, and a positive "Lupus Band Test" in both sun-exposed and non-exposed, non-lesional skin. What is the most likely diagnosis?

- a) Discoid Lupus Erythematosus
- b) Systemic Lupus Erythematosus
- c) Dermatomyositis
- d) Subacute Cutaneous Lupus Erythematosus

Q130. A 52-year-old male presents with a rapidly growing, painless, dome-shaped, firm, flesh-colored nodule with a central keratinous plug on his sun-damaged forearm. The lesion reached its current size of 2 cm within four weeks. Histopathology shows a well-differentiated squamous proliferation with a central crater filled with cornified debris and "glassy" eosinophilic cytoplasm in the surrounding keratinocytes. What is the most likely clinical course if this lesion is left untreated?

- a) Rapid metastatic spread to regional lymph nodes
- b) Spontaneous involution leaving a scarred area
- c) Transformation into a high-grade melanoma
- d) Progressive destruction of underlying bone and muscle

Q131. A 48-year-old male with a history of severe ethanol use disorder is admitted to the medical ward. Approximately 60 hours post-admission, he develops profound disorientation, hyperpyrexia, and significant autonomic hyperexcitability, including diaphoresis and tachyarrhythmia. The patient is observed engaging in complex, purposeless motor activities, appearing to grasp at non-existent filaments in the air. Physical examination reveals a coarse, high-frequency tremor and pupillary mydriasis. What is the pharmacological rationale for the first-line intervention in this patient?

- a) Competitive inhibition of aldehyde dehydrogenase to induce a formal aversion response
- b) Modulation of the GABA-A receptor complex to mitigate NMDA-mediated glutamate excitotoxicity



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- c) Antagonism of central D2 receptors to suppress dopaminergic hallucinosis
- d) Alpha-2 adrenergic agonism to suppress the sympathetic outflow from the locus coeruleus

Q132. A 29-year-old patient describes an experience where they perceive vivid, colorful geometric patterns superimposed on the neutral walls of the examination room. The patient insists these patterns are physically present and attempts to trace them with their finger. There is no corresponding external optical stimulus. The patient demonstrates a complete lack of insight, rejecting the clinician's suggestion that these visions are a product of their mind. This perception occurs in the absence of any transition between sleep and wakefulness. How is this phenomenon classified in clinical psychopathology?

- a) Pareidolia
- b) Sensory Deception (True Hallucination)
- c) Functional Hallucination
- d) Reflex Hallucination

Q133. An 11-year-old child is referred for evaluation due to profound deficits in social-emotional reciprocity. The parents note that the child has never engaged in proto-declarative pointing or joint attention. The child exhibits highly circumscribed interests in the longitudinal properties of train schedules and demonstrates motor stereotypies, such as repetitive digital posturing. There is a marked insistence on environmental sameness, where minor deviations in furniture arrangement precipitate catastrophic behavioral outbursts. What is the most likely neurodevelopmental diagnosis?

- a) Stereotypic Movement Disorder
- b) Autism Spectrum Disorder
- c) Schizoid Personality Disorder of Childhood
- d) Semantic-Pragmatic Language Disorder

Q134. A 40-year-old patient with a history of recurrent major depressive episodes presents in a state of profound psychomotor inhibition. The patient is mute, maintains a fixed posture for hours, and exhibits "waxy flexibility" (cerea flexibilitas) during the physical examination. The patient has ceased all nutritional intake, leading to clinical dehydration. Previous trials with multiple classes of second-generation antidepressants have failed to alleviate his mood symptoms. What is the most appropriate and rapid intervention to decrease morbidity and mortality?

- a) Initiation of a Monoamine Oxidase Inhibitor (MAOI) with a low-tyramine diet
- b) Intravenous administration of Ketamine
- c) Electroconvulsive Therapy (ECT)
- d) High-frequency Repetitive Transcranial Magnetic Stimulation (rTMS)

Q135. A 38-year-old female is evaluated for a severe depressive episode characterized by profound psychomotor inhibition. On examination, the patient exhibits "negativism" and "catalepsy," maintaining awkward postures for extended periods. She has developed significant prerenal azotemia due to a total cessation of oral fluid and nutritional intake. Her history is notable for failed therapeutic trials of three different classes of antidepressants at maximum tolerated doses. Given the presence of life-threatening somatic complications and the failure of pharmacological neuroplasticity induction, what is the most appropriate definitive intervention?

- a) Vagus Nerve Stimulation (VNS)
- b) Deep Brain Stimulation (DBS) of the subgenual cingulate cortex
- c) Electroconvulsive Therapy (ECT)
- d) Intravenous Esketamine infusion



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- Q136. A 55-year-old patient with a suspected malignant pulmonary nodule undergoes molecular imaging using a radiopharmaceutical that is a structural analogue of a hexose sugar. The tracer is transported into the intracellular compartment via GLUT-1 transporters and subsequently acted upon by hexokinase. However, the resulting metabolite cannot undergo further isomerization by phosphoglucose isomerase and becomes metabolically trapped. This phenomenon is an application of which of the following?
- The Warburg Effect
 - The Bohr Effect
 - The Haldane Effect
 - The Pasteur Effect
- Q137. A 62-year-old male presents with a productive cough and pyrexia. A posteroanterior chest radiograph reveals a dense opacity in the right hemithorax. The radiologist notes that the right cardiac border is sharply demarcated and clearly visible, but the lateral aspect of the right hemidiaphragm is no longer discernible. Applying the principle of contiguity and the loss of anatomical interfaces, where is the pathology localized?
- Right Middle Lobe (RML)
 - Right Lower Lobe (RLL), Superior Segment
 - Right Lower Lobe (RLL), Basal Segments
 - Apical Segment of the Right Upper Lobe
- Q138. A 22-year-old male is brought to the trauma bay following a high-velocity cranial impact. He initially regained consciousness but subsequently experienced a rapid decline in the Glasgow Coma Scale score. Non-contrast computed tomography (NCCT) of the head reveals a hyperdense, lenticular collection in the temporoparietal region that does not cross the coronal or lambdoid sutures. The mass effect has resulted in a midline shift. Which of the following vascular structures is most likely compromised?
- Bridging cortical veins
 - Middle meningeal artery
 - Internal carotid artery
 - Great cerebral vein of Galen
- Q139. A 10-year-old child presents with a tender, fluctuant mass over the frontal bone accompanied by periorbital edema and high-grade pyrexia. The patient has a history of chronic purulent rhinorrhea. NCCT of the paranasal sinuses demonstrates opacification of the frontal sinus with evidence of cortical erosion and a subperiosteal collection. What is the most likely diagnosis?
- Lemierre Syndrome
 - Pott's Puffy Tumor
 - Bezold Abscess
 - Orbital Cellulitis
- Q140. A 45-year-old male presents with a 5-day history of productive cough and pleuritic thoracic pain. Physical examination reveals tachypnea and localized crepitations. A posteroanterior chest radiograph demonstrates a homogeneous opacity in the right lower lung field. The radiologist notes that the right hemidiaphragm is obscured, but the right cardiac border remains sharply defined and clearly visible against the adjacent lung parenchyma. Based on the concept of anatomical contiguity and the loss of radiographic interfaces, which specific segment(s) are primarily involved?
- Lateral and medial segments of the Right Middle Lobe
 - Basal segments of the Right Lower Lobe
 - Superior segment of the Right Lower Lobe
 - Anterior segment of the Right Upper Lobe



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- Q141. A 45-year-old male presents with sudden-onset, excruciating epigastric pain that has become generalized. Physical examination reveals "board-like" abdominal rigidity and positive Rebound tenderness. An erect posteroanterior chest radiograph is obtained, revealing a crescentic hyperlucency positioned between the superior aspect of the hepatic shadow and the right hemidiaphragm. What is the most likely underlying mechanism for this radiographic finding?
- a) Interposition of the colon between the liver and diaphragm (Chilaiditi syndrome)
 - b) Escape of intraluminal gas into the peritoneal cavity following transmural necrosis
 - c) Accumulation of loculated pleural fluid in the subpulmonic space
 - d) Hyperinflation of the pulmonary parenchyma secondary to small airway obstruction
- Q142. A 12-year-old male presents with persistent, deep-seated nocturnal pain in the distal femur that is poorly responsive to non-steroidal anti-inflammatory drugs. Physical examination reveals a palpable, tender mass and localized warmth. A radiograph demonstrates a poorly defined lytic lesion in the metaphysis with a "sunburst" spiculation of bone and a triangular elevation of the periosteum. What is the most likely histopathological hallmark of this condition?
- a) Small round blue cells expressing CD99
 - b) Production of mineralized osteoid by malignant stromal cells
 - c) Proliferation of benign multinucleated giant cells
 - d) Cartilaginous cap with underlying bony stalk
- Q143. A 50-year-old male presents with vague flank discomfort. An ultrasound reveals significant dilatation of the renal pelvicalyceal system. A subsequent Contrast-Enhanced Computed Tomography (CECT) demonstrates a 6-mm hyperdensity (> 1000 Hounsfield Units) lodged at the ureteropelvic junction (UPJ). What is the most common pathological sequence leading to this radiographic presentation?
- a) Retroperitoneal fibrosis causing extrinsic compression
 - b) Luminal transition of a crystalline aggregate causing obstructive uropathy
 - c) Neoplastic transformation of the transitional epithelium
 - d) Ascending pyelogenic infection leading to papillary necrosis
- Q144. A hospital radiation safety officer is reviewing the safety protocols for a new interventional radiology suite. He emphasizes that while certain effects of radiation, such as erythema and epilation, occur only after exceeding a specific dose threshold, other risks, such as the induction of leukemia, follow a linear non-threshold (LNT) model. Which of the following terms correctly classifies the latter risk?
- a) Deterministic effect
 - b) Stochastic effect
 - c) Hormetic effect
 - d) Teratogenic effect
- Q145. A patient in the intensive care unit undergoes aggressive volume expansion with isotonic crystalloids following a septic episode. Over the next 24 hours, the patient develops diffuse soft-tissue swelling and a significant positive fluid balance. Laboratory analysis reveals a decrease in serum albumin concentration due to dilutional effects. According to the Starling equation, which of the following best describes the physiological shift primarily responsible for the resulting anasarca?
- a) Elevation of the interstitial oncotic pressure (π_i)
 - b) Reduction of the plasma colloid oncotic pressure (π_p) coupled with increased capillary hydrostatic pressure (P_c)
 - c) Decrease in the capillary filtration coefficient (K_f)



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d) Increase in the reflection coefficient (σ) of the vascular endothelium

Q146. A 28-week-gestation neonate, stabilized on mechanical ventilation with a high fraction of inspired oxygen (FiO_2 FiO_2), is noted to have abnormal retinal findings during a longitudinal screening. The ophthalmologist describes an initial phase of vaso-oblivation followed by an aggressive phase of disordered angiogenesis at the junction of vascularized and non-vascularized retina. Which of the following molecular mediators is most directly upregulated during the secondary hypoxic phase of this pathology?

- a) Transforming Growth Factor-beta ($TGF-\beta\beta$)
- b) Vascular Endothelial Growth Factor (VEGF)
- c) Retinol-binding protein 4 (RBP4)
- d) Endostatin

Q147. During the administration of regional anesthesia for an obstetric procedure, the clinician utilizes the "loss of resistance" technique to identify a specific anatomical compartment. The needle tip is positioned posterior to the dura mater and anterior to the ligamentum flavum. Which of the following anatomical structures is contained within this target compartment?

- a) Cerebrospinal fluid and the filum terminale
- b) Internal vertebral venous plexus and adipose tissue
- c) Denticulate ligaments and the conus medullaris
- d) Anterior spinal artery and the pia mater

Q148. Shortly after the infiltration of a high-volume amide-type local anesthetic for a peripheral nerve block, a patient reports an unusual gustatory sensation described as "metallic" and persistent "ringing" in the ears. Within minutes, the patient exhibits involuntary muscular twitching and progresses to a generalized tonic-clonic seizure. What is the primary molecular mechanism responsible for this systemic toxicity?

- a) Competitive antagonism of muscarinic acetylcholine receptors
- b) Non-selective blockade of voltage-gated sodium channels in the CNS
- c) Potentiation of GABAergic inhibitory neurotransmission
- d) Activation of N-methyl-D-aspartate (NMDA) receptors

Q149. A patient requires urgent reversal of a deep neuromuscular blockade induced by rocuronium. The clinician chooses a pharmacological agent that acts as a "selective relaxant binding agent" (SRBA). This molecule is a modified γ -cyclodextrin with a hydrophobic core that exerts its effect by forming a 1:1 guest-host inclusion complex with the steroid nucleus of the paralytic agent. What is a unique clinical advantage of this mechanism over traditional acetylcholinesterase inhibitors?

- a) It avoids the need for co-administration of antimuscarinic agents
- b) It increases the half-life of the neuromuscular blocker in the plasma
- c) It acts by increasing the concentration of acetylcholine at the nicotinic receptor
- d) It is equally effective for both aminosteroid and benzylisoquinolone compounds

Q150. Six members of the same family are brought to the emergency department with abrupt behavioral changes. About 1 hour earlier, they consumed a homemade herbal decoction purchased from an apothecary. All are flushed, tachycardic, febrile, and markedly agitated. Several are reaching for imaginary objects, picking at bed sheets, and trying to throw objects through a closed window. Examination shows dry mucous membranes, dry axillae, dilated pupils, reduced bowel sounds, and urinary retention. There is no diarrhea, lacrimation, bronchorrhea, diaphoresis, muscle fasciculation, or miosis. ECG shows sinus tachycardia



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without QRS widening. The physician suspects poisoning by a plant-derived tropane alkaloid with atropine-like activity. All of the following statements are true regarding this condition except:

- a) The toxidrome is due to blockade of muscarinic acetylcholine receptors, and plant alkaloids such as atropine, hyoscyamine, and scopolamine can produce this syndrome.
- b) Dry skin, dry mucosa, mydriasis, delirium, hyperthermia, urinary retention, and decreased bowel sounds support anticholinergic toxicity.
- c) The most appropriate antidotal strategy is additional atropine plus pralidoxime because the primary mechanism is irreversible acetylcholinesterase inhibition.
- d) Management includes supportive care, cooling, benzodiazepines for agitation, and selected use of physostigmine in severe pure anticholinergic poisoning.

ANSWERS

1	b	31	c	61	b	91	c	121	b
2	c	32	b	62	b	92	b	122	b
3	c	33	b	63	c	93	b	123	b
4	b	34	b	64	a	94	b	124	c
5	b	35	c	65	c	95	b	125	c
6	c	36	c	66	c	96	b	126	b
7	c	37	c	67	b	97	b	127	c
8	b	38	b	68	c	98	b	128	a
9	c	39	c	69	b	99	b	129	b
10	c	40	b	70	b	100	c	130	b
11	b	41	a	71	d	101	a	131	b
12	a	42	b	72	b	102	c	132	b
13	b	43	b	73	b	103	b	133	b
14	a	44	b	74	a	104	c	134	c
15	d	45	c	75	c	105	b	135	c
16	b	46	c	76	b	106	a	136	a
17	c	47	b	77	b	107	c	137	c
18	c	48	c	78	b	108	b	138	b
19	c	49	c	79	b	109	c	139	b
20	b	50	b	80	c	110	b	140	b
21	c	51	c	81	b	111	a	141	b
22	a	52	b	82	b	112	b	142	b
23	b	53	c	83	b	113	b	143	b
24	b	54	c	84	b	114	b	144	b
25	c	55	c	85	b	115	b	145	b
26	a	56	a	86	b	116	c	146	b
27	b	57	b	87	b	117	b	147	b
28	b	58	b	88	b	118	a	148	b
29	b	59	b	89	b	119	c	149	a
30	b	60	c	90	c	120	b	150	c



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